

The Global Burden of Mental Illness:

Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment.

Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major Depression

1 in 3

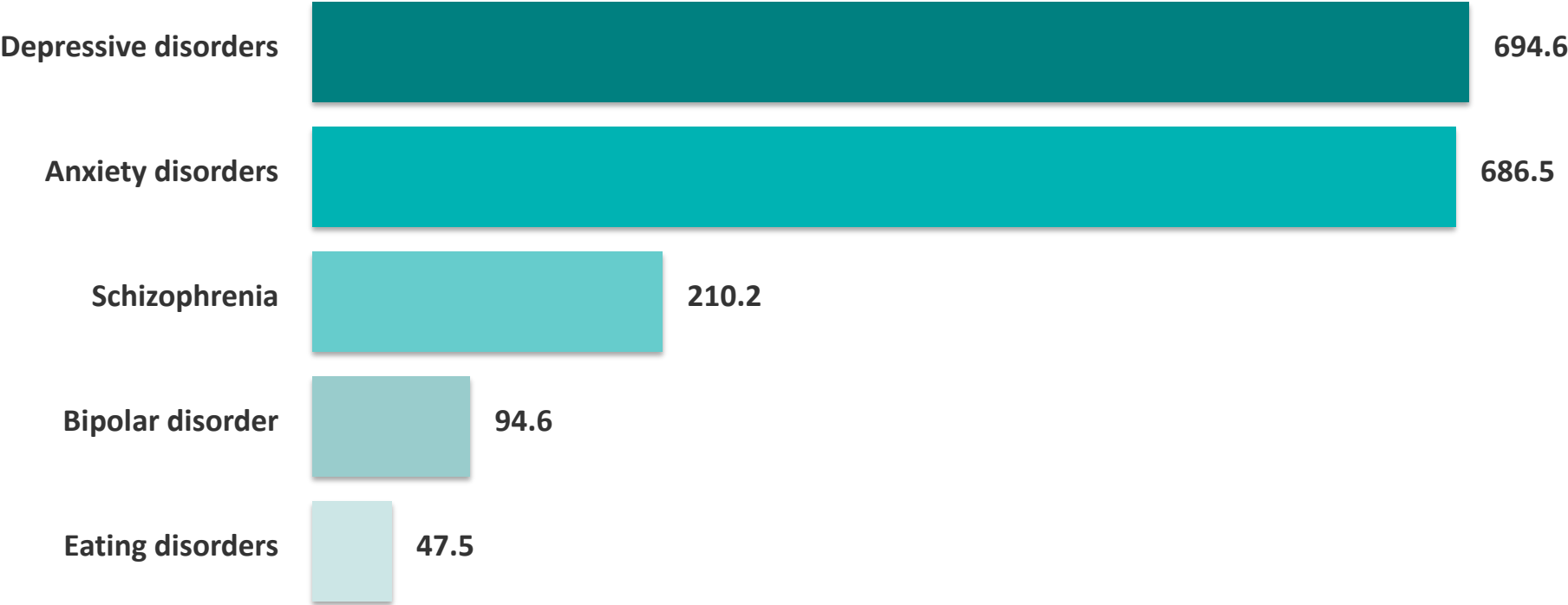
women will experience major
depression in their lifetime

1 in 5

men will experience major depression
in their lifetime

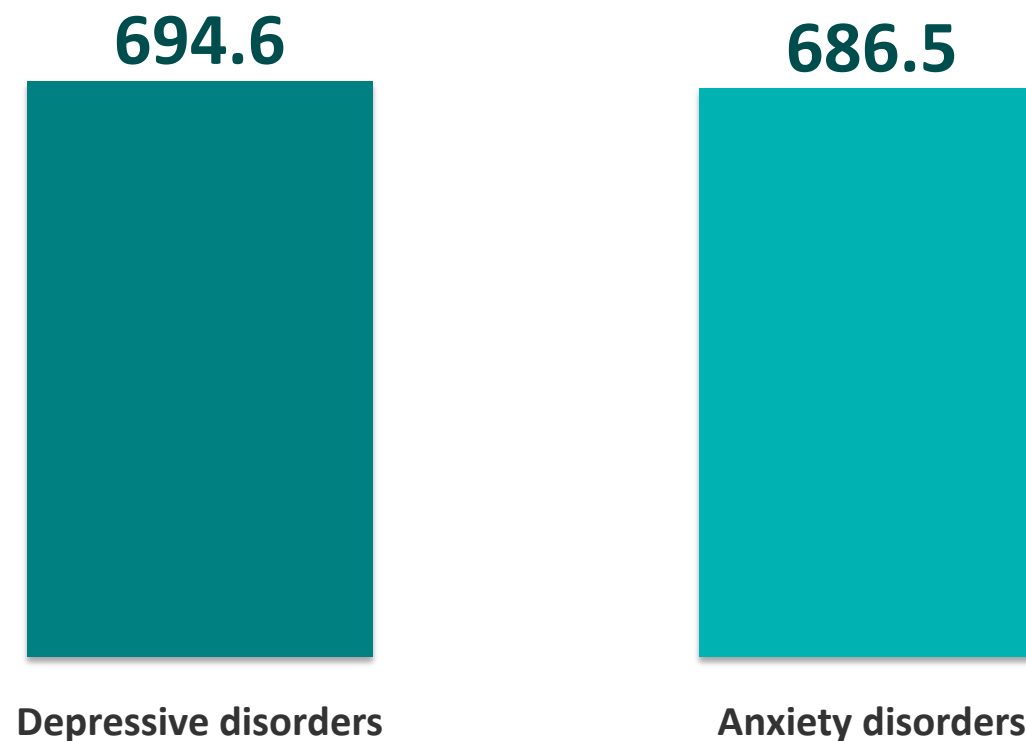
Hundreds of millions of people suffer from mental health conditions each year. Conditions range from common disorders (depression and anxiety) to less common but severe illnesses (schizophrenia and bipolar disorder). Mental illnesses are treatable, yet treatment is often lacking or of poor quality, and stigma prevents many from seeking help.

Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respectively



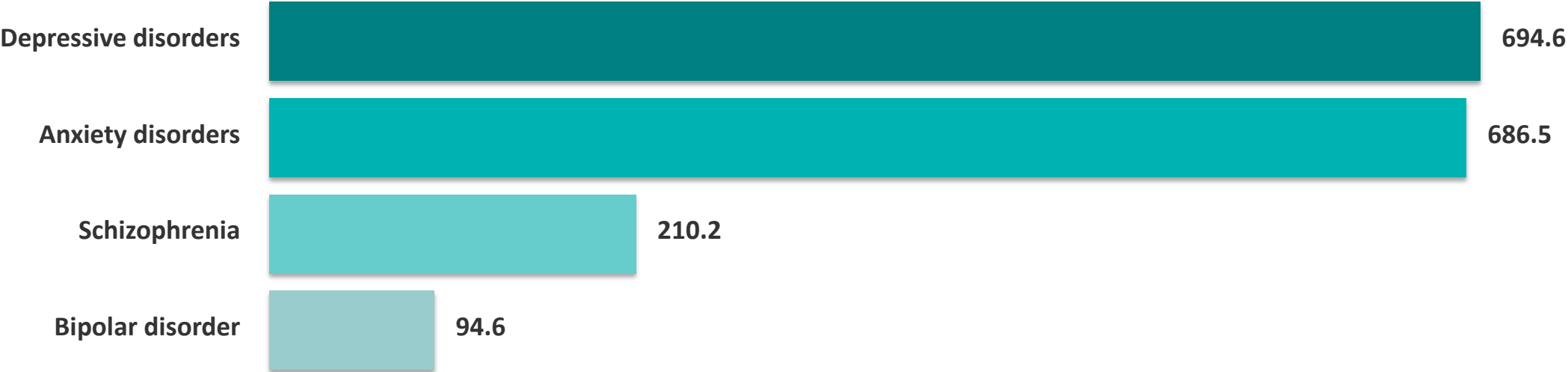
Disease burden measured in DALYs per 100,000 people | Source: IHME, Global Burden of Disease (2023)

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



Together, depressive and anxiety disorders account for roughly 80% of the total mental health disease burden across these five categories. Their near-equal DALY rates (~695 and ~687) underscore that both conditions demand comparable policy attention and resource allocation.

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact



Key Insight

While schizophrenia and bipolar disorder contribute smaller shares at the population level, their individual severity and disability are high. Schizophrenia alone contributes 210.2 DALYs per 100,000 — reflecting the profound impact on those affected. Policy must balance population-scale interventions with specialized, sustained care for severe conditions.

Treatment Is Available but Often Lacking or Poor Quality



S

Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.



A

Access

Treatment is often lacking, especially in low- and middle-income countries.



Q

Quality

Even where treatment exists, it may be of poor quality.



U

Underreporting

Discomfort with disclosure makes it difficult to estimate true prevalence — the actual burden may be higher.

How People Cope: From Medication to Faith to Family Conversations

Prescribed Medication

Common in higher-income settings as a primary response to anxiety and depression.

Religious or Spiritual Activities

Prevalent in many regions worldwide, reflecting the role of faith communities in mental health support.

Talking to Friends or Family

A widely reported coping mechanism across cultures and income levels.

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

1. Depression and anxiety together represent the overwhelming majority of mental health DALYs — policy should prioritize scaling evidence-based interventions for these common disorders.
2. Schizophrenia and bipolar disorder carry disproportionate individual burden; specialized, sustained care for severe conditions must be maintained.
3. Effective treatments exist, but access and quality remain inadequate — especially in low- and middle-income countries.
4. Stigma suppresses both help-seeking and accurate data, meaning the true burden is likely underestimated.
5. Coping strategies vary widely across cultures; successful policies must be culturally sensitive and context-specific.